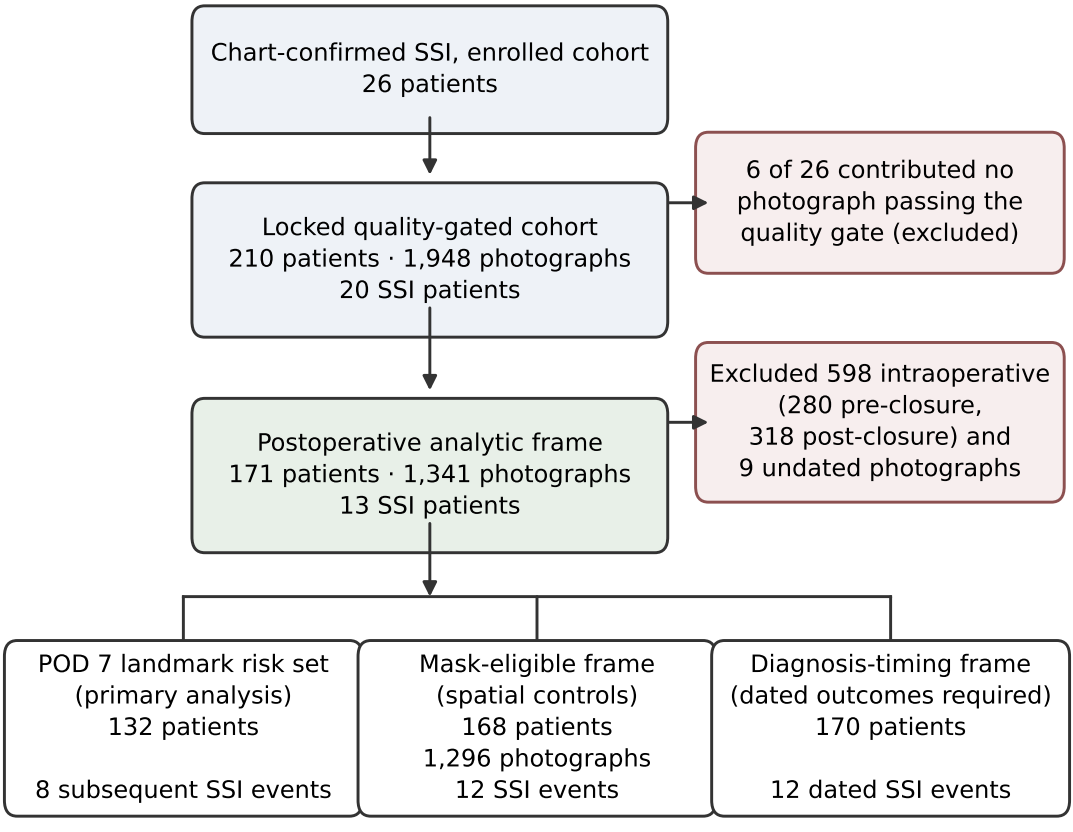

Event counts differ by analysis because each frame imposes a different requirement: the POD 7 landmark excludes patients already diagnosed at the landmark and those without a recorded diagnosis day; the mask-eligible frame requires a usable incision annotation; the diagnosis-timing frame requires a dated outcome.

Attrition at the quality gate is outcome-dependent and its direction is not known: a packed or vacuum-managed wound is harder to photograph and may be the most obviously infected, so excluding it could lower performance as readily as raise it. Every estimate is conditioned on it.